

Laparoscopic Sleeve Gastrectomy

Surgical Case Series Report

Retrospective Analysis • January 2023 – December 2024 • n=18 Patients

PRINCIPAL SURGEON

Dr. Amelia Voss, FRCS

Consultant Bariatric Surgeon

CO-INVESTIGATOR

Dr. Carlos Mendez, MD

Surgical Registrar

INSTITUTION

Meridian Academic Medical Centre

14 Harrow Lane, Westfield, WF2 9KT

ETHICS REFERENCE

MAMC-SURG-2023-0041

Approved by IRB Committee

SUBMISSION DATE

15 February 2025

REPORT VERSION

v2.1 – Final

FUNDING

No external funding received

Abstract. This retrospective case series evaluates the surgical outcomes and short-term complications of laparoscopic sleeve gastrectomy (LSG) performed at Meridian Academic Medical Centre between January 2023 and December 2024. Eighteen consecutive patients (mean age 41.6 ± 9.2 years; mean preoperative BMI 46.3 ± 7.1 kg/m²) underwent standard five-port LSG under general anaesthesia. The primary endpoint was percentage excess weight loss (%EWL) at 12 months. Secondary endpoints included 30-day morbidity, operative time, length of hospital stay, and resolution of obesity-related comorbidities. Mean %EWL at 12 months was $68.4 \pm 12.7\%$. No intraoperative deaths occurred. Major complications (Clavien–Dindo grade $\geq$ III) were observed in 2/18 patients (11.1%). These findings support LSG as a safe and effective bariatric procedure in a high-volume academic centre.

Keywords: sleeve gastrectomy; bariatric surgery; obesity; weight loss; morbidity; case series

INTRODUCTION

Obesity remains a global health challenge, affecting approximately 650 million adults worldwide and increasing the risk of type 2 diabetes, hypertension, obstructive sleep apnoea, and cardiovascular disease. Metabolic and bariatric surgery is the most effective long-term treatment for severe obesity, achieving sustained weight loss and significant comorbidity resolution.

Laparoscopic sleeve gastrectomy has emerged as the most commonly performed bariatric procedure globally, accounting for roughly 60% of all primary bariatric operations. Its relative technical simplicity, favourable risk profile, and consistent efficacy make it particularly suitable for high-volume centres seeking standardised patient outcomes. This report presents a retrospective case series of 18 consecutive LSG procedures conducted at Meridian Academic Medical Centre, with the aim of documenting operative parameters, perioperative outcomes, and 12-month weight-loss data.